

PREPARED FOR BELFIORE MEDICAL AESTHETICS

The Client Experience Programme

Two days with the front-of-house team, and the system that makes them hold

- 01 What good looks like here
- 02 Doing the job day to day
- 03 When it is difficult
- 04 The client who is not in the room
- 05 How we know it is working

PREPARED FOR

Dr Uju Rapu, Managing Director and Chief Executive, Belfiore Medical Aesthetics

DELIVERY

25 and 26 September 2026, five hours each day

CLASSIFICATION

Confidential to Belfiore Medical Aesthetics

PREPARED BY

Dr Debo Odulana, Founding Partner, Consult for Africa

PARTICIPANTS

Three, from the administrative and front-of-house team

DURATION

2 days

25 and 26 September, five hours a day

TAUGHT SESSIONS

10

across five domains, nothing taught twice

LIVE SURVEYS

4

run before the first session

PROGRAMME

The Client Experience Programme, built for a medical aesthetics practice

AUDIENCE

The Belfiore administrative and front-of-house team

SHAPE

Five domains, ten sessions, roughly half of the time spent practising

BEFORE

A baseline diagnostic: four short surveys, live now

AFTER

A signed thirty-day plan for each participant

INVESTMENT

N600,000 for three participants, exclusive of VAT

A letter to Dr Rapu

Dear Dr Rapu,

You asked us for customer service training for your admin staff. What follows is that, and a little more than that, for the same two days.

The reason is simple. In a medical aesthetics practice the front desk is not an administrative function. It is the commercial one. A woman who has decided she wants a treatment on her face has already done the hardest part of your marketing for you. What happens between that decision and her appointment is almost entirely in the hands of the person who answers the phone, the WhatsApp message or the Instagram enquiry. Whether she books, what she pays, whether she comes back in twelve weeks and whether she tells three friends are all decided at the desk, not in the treatment room.

So a generic customer service course would be a waste of two days of your team's time and your money. What your people need is not a reminder to smile. They need to know what to say when a client asks the price and expects a discount, what to do when somebody is unhappy with a result, what they are allowed to decide without coming to you, and who calls a client back at twelve weeks and on what list.

There is a second thing I want to be straight with you about. **Training on its own decays.** Two good days will lift behaviour for about six weeks and then the desk returns to whatever it did before, because nothing changed except what three people know. What makes it permanent is a written standard, a way of measuring whether the standard is being met, and somebody whose job it is to notice. Those three things are not in the two days, and rather than quietly leave them out and call the two days a complete solution, I have set out what they would involve in an appendix at the back. It is there for later. It is not what you asked for and it is not what this document is about.

Before we teach anything, we want to know what is actually happening at your desk. Four short surveys go out with this document: one for your team, which is anonymous and is the one that matters most, one for your clients, one for the people who enquired and never came in, and one for you and your senior

colleagues. What comes back sets the content of the two days.

We are already building your client record system, so we come to this with some sight of how your information moves. This programme is about the people who use it.

Thank you for trusting us with your team.

Dr Debo Odulana

Founding Partner, Consult for Africa

01 What this programme is actually for

Three commercial facts sit behind the design. They are the reason the syllabus looks the way it does.

Your enquiry channels are your sales floor. In aesthetics, first contact arrives by Instagram message, WhatsApp and phone, at all hours, from someone who is comparing you against two or three other clinics in the same fifteen minutes. Response speed is not a courtesy. It is the largest single lever on conversion that a practice of your size controls, and it costs nothing to move.

Your revenue is a repeat revenue. Injectables recur at twelve to sixteen weeks. Skin programmes recur at four to six. Laser is sold as a course. A client who is not called back is not a lost visit, she is a lost year. Recall is not marketing, it is the core operating rhythm of the business, and it belongs to the front desk.

Your risk is reputational and it is personal. Aesthetics deals with a client's face, at her own expense, with an outcome she judges subjectively in a mirror. When she is disappointed she is disappointed in a way a general practice patient rarely is, and she has an audience. How the desk handles that first hour decides whether it becomes a conversation or a post.

Everything in the two days serves one of those three.

02 How the programme is built

Six principles shape it.

- **Objectives first, content second.** Every session is defined by what participants will be *able to do*, not by what will be covered. That is what makes it assessable, and it is what lets you hold the team to it afterwards.
- **Mutually exclusive, collectively exhaustive.** The whole of client experience is divided into five domains. No topic sits in two of them, and together they cover everything that happens around a client. Nothing is taught twice, and nothing that matters is left out.
- **Built on Belfiore, not on a case study.** The role plays are your treatments, your price points, your channels and the situations your team actually meets. The baseline surveys exist so that this is true rather than claimed.
- **Every session names the money.** Each one states which of the three commercial facts above it protects, so the team understands service as the revenue function it is rather than as politeness.
- **Practised, not presented.** Roughly half of the contact time is exercise, role play and live rework of your own messages and scripts.

- **Measured, not assumed.** There is a baseline before the training and an audit after it. Attendance is not evidence that anything changed.

03 The frame: five domains

Client experience decomposes cleanly into five domains. The test that separates them is simple and it holds: **before she arrives, while she is with you, when something has gone wrong, after she leaves, and how you know any of it is happening.**

#	Domain	The question it answers
01	What good looks like here	What are we holding ourselves to?
02	Doing the job day to day	How do we handle a client whose visit is going well?
03	When it is difficult	How do we handle one whose visit is not?
04	The client who is not in the room	What happens between visits?
05	How we know it is working	Who checks, how often, and against what?

Ten sessions, five hours a day, two days. Every one of the ten sits in exactly one domain. If a question about a client cannot be placed in one of these five, it is not a client experience question.

04 Day one

Domains 01, 02 and the first half of 03. Understanding what good looks like, and doing it.

Session 1. Where the money is: the Belfiore client journey

Domain 01. Protects: conversion.

We map, on the wall, every point at which a person meets Belfiore, from seeing a post to being called back a year later. Then we mark the four or five moments that actually decide whether she proceeds, returns and refers, because they are not the ones most practices spend their effort on.

By the end, participants can describe the Belfiore client journey end to end; name the moments where a client is most likely to be lost; and say what their own role does at each one.

Practical application: journey mapping with your own enquiry threads from the last month.

Session 2. The Belfiore service standard

Domain 01. Protects: all three.

You cannot train against nothing, coach against nothing or audit against nothing. The team drafts the standard it will be held to: reply times by channel, who owns an enquiry, what may be decided at the desk and what must be escalated, what is promised and what is never promised.

By the end, participants can state the response-time standard for every channel; say what they are authorised to decide alone; and identify what must be escalated and to whom.

Practical application: a drafted one-page Belfiore service standard, to come to you for sign-off.

Session 3. First contact on every channel

Domain 02. Protects: conversion.

The enquiry as it actually arrives: an Instagram message at 11pm, a WhatsApp voice note, a phone call during a treatment, a walk-in while two people are already waiting. How to open, how to qualify without interrogating, how to move somebody from a channel into a booked consultation, and how to answer "**how much is it**" without either deflecting or collapsing the price.

By the end, participants can run a first contact on any of your four channels to a booked consultation or a recorded outcome; answer a price question with confidence; and handle three or four enquiries at once without losing one.

Practical application: live rework of your own recent enquiry threads, then role play on each channel.

Session 4. In the clinic, from the door to the till

Domain 02. Protects: retention and reputation.

Arrival, the wait, the reception area as a private space, the handover to the clinician, the checkout, the payment conversation and the next appointment. The small mechanics that decide whether a first visit feels like a clinic or like a shop.

By the end, participants can run an arrival and a checkout to the standard; tell a waiting client what is happening before she has to ask; and close a visit with the next appointment booked.

Practical application: a walk-through of your own reception, timed, with the gaps written down.

Session 5. Confidentiality, consent and discretion

Domain 02. Protects: reputation and your legal position.

The part a general customer service course does not contain and an aesthetics practice cannot do without. What the Nigeria Data Protection Act 2023 requires of you in plain language. Photographs: why consent for the clinical record and consent for marketing are two different permissions, both written, both withdrawable. Personal phones and WhatsApp groups. What may be said at a desk where another client can hear. And the one your team will meet sooner than you think: a caller asking whether a named person is a client here, and why the answer is the same whoever is asking.

By the end, participants can state what may and may not be disclosed and to whom; take and record photographic consent correctly; handle a request to confirm a client's attendance; and keep a front-desk conversation private in a small reception.

Practical application: eight real scenarios, decided in pairs, then argued out.

05 Day two

The rest of domain 03, then 04 and 05. When it is hard, what happens afterwards, and how it is held in place.

Session 6. The difficult client

Domain 03. Protects: reputation and retention.

A difficult client is not one thing, and treating her as one thing is why most service training fails at exactly this point. There are six, they are driven by six different things, and the response that works for one makes another worse. Section 6 of this document sets them out in full.

By the end, participants can identify which of the six is in front of them within the first minute; select the matching response; hold a boundary without losing warmth; and recover their own composure afterwards.

Practical application: six role plays, one per type, drawn from situations your team reports in the baseline survey.

Session 7. The dissatisfied result

Domain 03. Protects: reputation and your clinical governance.

The situation specific to your business: a client who is unhappy with how she looks. It is emotional, it is often clinical, and the front desk is the first to meet it. The rule is precise. **Admin never assesses a clinical outcome and never concedes one.** What the desk does is acknowledge, escalate within a stated time, record accurately, and keep the client with you while the clinician deals with it.

By the end, participants can take a result complaint without diagnosing, defending or conceding; escalate it to the right clinician within the standard; record it so it cannot be lost; and manage the hours in between so the client does not go public while she waits.

Practical application: the complaint-response framework, then three escalations run live.

Session 8. Follow-up and aftercare contact

Domain 04. Protects: retention and reputation.

What happens in the days after a treatment, and who owns it. The twenty-four hour check, the seven-day check, the aftercare instructions that were given verbally and forgotten before she reached her car, the review request timed to when she can actually see the result rather than when it suits you, and the referral ask made at the only moment it ever works.

By the end, participants can run the aftercare contact sequence; write a check-in message that gets a reply; ask for a review and a referral at the right moment; and recognise in a reply the early sign of a complaint.

Practical application: the team writes the actual message set, which becomes yours to keep.

Session 9. Prompting, recall and rebooking

Domain 04. Protects: revenue, directly.

The commercial engine of the practice and the part most clinics never systematise. Confirmations, the reminder at seventy-two hours and again at twenty-four, deposits and what they are really for, the recall interval for each of your treatment lines, the rebook offered at checkout rather than chased three months later, and the reactivation of clients nobody has contacted in a year.

By the end, participants can operate the recall list; state the return interval for each treatment line; offer the next appointment at checkout; run a reactivation contact; and explain a deposit without apologising for it.

Practical application: building your recall calendar, treatment by treatment, and drafting the reactivation message.

Session 10. Feedback, audit and making it stick

Domain 05. Protects: all three, permanently.

How you find out whether any of this is happening when nobody is watching. The feedback instrument and when to send it. Mystery shopping your own front desk. Reviewing calls and messages as a coaching act rather than a disciplinary one. The five numbers worth looking at every week. Then the mechanics of permanence: the fifteen-minute weekly huddle, who owns client experience by name, and each participant's signed thirty-day plan.

By the end, participants can run the weekly service huddle; read the five numbers and say what each one means; receive a review of their own call without defensiveness; and state their own thirty-day commitments.

Practical application: each participant writes and signs a thirty-day plan. You receive a copy.

06 The six difficult clients

This is the section your team will use most, so it is set out in full. The six are distinguished by **what is driving the behaviour**, not by how loud it is. That is what makes the set usable under pressure: one question, asked silently, tells the person at the desk which response to reach for.

The client	What is driving her	What she needs first	The mistake that makes it worse
The anxious one	Fear, not anger. She is about to let someone change her face.	Time, information and an unhurried voice	Efficiency. Being handled quickly reads as being dismissed
The angry one	A specific failure: a wait, a cancellation, a charge, a botched booking	Acknowledgement of the failure before any explanation	Explaining first. An explanation before an acknowledgement is heard as an excuse
The disappointed one	A gap between what she expected and what she sees in the mirror	To be taken seriously and moved to the clinician fast	Reassurance from the desk. It is not yours to give and it destroys the clinician's position
The entitled one	Status. She expects to be an exception	To be recognised, then given a warm and immovable boundary	Bending the rule. The exception becomes the new baseline and the team loses the standard
The negotiator	Money. She wants a discount and will test for one	Value restated in terms of what she is actually buying	Conceding at the desk, or apologising for the price
The public one	An audience. She feels she has to be loud to be heard	Speed, a named person, and a move off the public channel	Arguing in public, deleting the comment, or going silent

Three rules that run across all six.

- 1. Acknowledge before you explain.** In every one of the six, the first move is the same and almost everyone gets it wrong under pressure.
- 2. Name yourself and own it.** "I am Chidinma, I am looking after this, I will come back to you by four o'clock" resolves more than any script. A client who has a name escalates far less often than one who has a department.
- 3. Never concede a clinical judgement and never concede a price at the desk.** Both belong to somebody else, and conceding either creates a problem nobody at the desk can undo.

07 The service standard you will end up with

This is the shape of what the team drafts in session 2 and you sign. It is not imposed. A standard the team wrote is a standard the team keeps.

Area	The standard
First response, opening hours	Instagram and WhatsApp within 30 minutes, never beyond 3 hours. Missed calls returned within 60 minutes. Email the same working day
First response, out of hours	An acknowledgement on receipt, a real reply by 10am
Ownership	Every enquiry has one named person until it is closed. Nothing sits in a shared inbox unowned
Outcome	No enquiry closes without a recorded outcome: booked, declined, or a reason
Price	The price is given when asked. It is never deflected to "let me check and revert" unless it genuinely varies
Waiting	No client waits more than 15 minutes without being told why and given a new time
Privacy	Nothing about a client is said where another client can hear. Attendance is never confirmed to a caller
Complaints	Logged the day they happen. A result complaint reaches a clinician within the stated time, always
Aftercare	Every treated client is contacted within 24 hours by a named person
Next appointment	No client leaves without knowing when to come back, and ideally with the appointment made

08 Measurement: the five numbers

You cannot hold a standard you are not measuring. Five numbers, reviewed weekly, are enough for a practice your size. They take one person about twenty minutes a week once the sheet exists.

#	The number	Why it is on the list
1	Median first response time, by channel	The single largest controllable lever on conversion, and the one that slips first when the desk is busy
2	Enquiries to booked consultations	Separates a demand problem from a conversion problem. They have opposite answers and opposite costs
3	Consultations to treatments	Tests the consultation itself, and whether expectations are being set honestly
4	Rebook rate at checkout	The cheapest revenue in the business. A rebook made at the till costs nothing; the same booking chased in March costs a campaign
5	No-shows and late cancellations	Directly measures whether prompting and deposits are working, and it is pure lost capacity

Two watch items alongside them: complaints logged and closed within seventy-two hours, and dormant clients reactivated in the month.

And the audit. Numbers tell you what happened, not why. A mystery shop, meaning contacts made to your own desk across all four channels by someone your team does not know, tells you what a stranger actually experiences. It is uncomfortable the first time and it is the most useful single input to coaching that exists.

09 The follow-up and recall calendar

Session 9 builds this with your team, against your own treatment lines. The shape is this.

When	What happens	Who
On booking	Confirmation with preparation notes and the deposit position	Front desk
72 hours before	First reminder	Automated, checked
24 hours before	Second reminder, with a real reply channel	Automated, checked
At checkout	Aftercare given in writing, next appointment offered	Front desk
24 hours after	Aftercare check-in from a named person	Front desk
7 days after	How are you feeling, anything you want us to look at	Front desk
At the result point	Review request and, where appropriate, the referral ask	Front desk
At the recall interval	Recall contact, by treatment line	Recall list
6 months quiet	Reactivation message	Recall list
12 months quiet	A call from a named person, not a message	Practice manager

The recall intervals themselves are clinical and they are yours to set. What we install is the discipline that somebody is on a list, the list is worked, and nobody falls off it because the person who remembered her has gone on leave.

10 Before, during and after

The two days sit inside a short arc. The arc is what makes the days work.

Before: the baseline. Four short surveys, live on a private page, going out with this document.

Survey	Who it is for	Why
The front desk, from the inside	Your team. Anonymous	What is genuinely hard right now: volume, tools, authority, the questions they cannot answer. This one sets the content of the two days
Your experience of Belfiore	Clients who have been treated. Anonymous	What the journey feels like from outside, including the parts nobody says to your face
You got in touch, and did not book	People who enquired and never came. Anonymous	The most commercially useful instrument on the list, and the one almost no clinic ever runs
The view from the top	You and your senior colleagues. Attributed	Run on the same five domains as the team survey, so the gap between what leadership believes and what the desk experiences becomes visible instead of assumed

During: the two days. Ten sessions, five hours a day, roughly half of it practice.

After: the thirty days. Each participant leaves with a signed plan and you receive a copy of all three. The weekly huddle starts in week one, run by your own team using the pack they build in session 10. You will be able to see for yourself, from the five numbers in section 8, whether it took.

11 What it costs

N600,000, for three participants, at N200,000 each. Additional participants are charged at N200,000. Fees are exclusive of VAT at 7.5%.

What the fee includes

Included	What that means
Programme design	The syllabus customised to your treatments, your channels, your price points and your client base, rather than a standard course delivered at you
The baseline diagnostic	Four live surveys, run before the first session, so the two days are built on what is actually happening at your desk
Ten taught sessions	Five hours a day across two days, roughly half of it practice, role play and live rework of your own messages
The materials	Participant workbook, the six difficult clients, the complaint-response framework, the aftercare message set and the recall calendar, all yours to keep
The service standard	Drafted by your team in session 2 and brought to you for sign-off. One page, yours to run
The thirty-day plans	Signed by each participant. You receive a copy of all three
Certificates	For each participant
Delivery	At our office in Lekki Phase 1, or at Belfiore by arrangement

Terms

Fees are invoiced in advance and are **non-refundable**, in whole or in part, including where a participant does not attend. A participant may be **substituted at any time at no charge**, so if somebody cannot make it, send a colleague. The dates may be **moved once at no charge** on five working days of written notice; inside five working days a rescheduled programme is charged at 50% of the fee and the original fee remains due. Seats are confirmed on receipt of payment.

12 Next steps

- 1. Settle the invoice** so the seats are confirmed. It is attached, and payment is due before the first session on 25 September.
- 2. Send the team survey to your three participants today.** It takes eight minutes and it is what makes the two days about Belfiore rather than about aesthetics in general.
- 3. Send the client and enquirer surveys** to whoever you are comfortable sending them to. The enquirer list is the valuable one, and it is sitting in your Instagram inbox.
- 4. Complete the leadership survey yourself**, and ask your senior colleagues to do the same.
- 5. Tell us where you want the training held**, at our office in Lekki Phase 1 or at Belfiore.
- 6. Tell us the three names**, so the workbooks and certificates carry them.

Appendix. If you ever want to go further

This appendix is not part of what you have asked for and nothing in it is needed for the two days to be worth doing. It is here because we would rather be straight with you about what it takes to make a service standard permanent than let you discover it in six months.

Two good days will lift behaviour for about six weeks. What makes the lift permanent is a written standard, a way of measuring whether it is being met, and somebody whose job it is to notice. The standard comes out of session 2 either way. The other two do not, and this is what they would involve.

Option B. The two days and the system

The increment, priced as resourced consultancy days at CFA's published rates.

Workstream	Grade	No. of days	Day rate	Amount
Mystery-shop baseline: eight contacts across four channels, plus a review of thirty recent enquiry threads	Consultant	2.0	N190,000	N380,000
The written service standard, drafted with the team, tested and signed off with you	Senior consultant	1.5	N300,000	N450,000
Feedback instrument, the weekly dashboard and the huddle pack	Consultant	1.5	N190,000	N285,000
The coaching cadence, built with whoever will own client experience by name	Senior consultant	1.0	N300,000	N300,000
Day-30 re-shop and the audit report, measured against the baseline	Consultant	1.5	N190,000	N285,000
The five numbers, and the read-out to you	Partner	0.5	N750,000	N375,000
Subtotal		8.0		N2,075,000
Programme rate, 10%, because this is one continuous mandate				(N207,500)
The increment				N1,867,500

With the two days, **N2,467,500** in total.

Option C. Adding a quarter of reinforcement

Option B plus three months: a monthly mystery shop, a monthly coaching session with whoever owns client experience, a quarter-end report and a session with you to reset the standard. **N3,245,000** in total.

The one timing point worth knowing now

If you ever want the measurement, the mystery-shop baseline has to be taken **before** the team is trained. Afterwards there is nothing to measure the change against. That is the only thing about this appendix that is time-sensitive, and it is the reason it appears in a document about something else.

All figures exclude VAT at 7.5%.

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